

Caregiver Starter Guide

English

Person before diagnosis.
Dignity before compliance.
Relationship before activity.
Engagement before completion.

Small changes in communication can change the interaction

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1. Understand

Dementia-related changes can affect comprehension, expression, initiation, memory, and processing speed. Hearing, vision, pain, fatigue, medication, environment, culture, and language also shape communication.

2. Prepare

Approach calmly, gain attention, use the preferred name, speak adult-to-adult, present one idea or instruction, allow time, and preserve choice.

3. Connect

During ordinary care, explain before acting, show as well as tell, break tasks into manageable steps, validate refusal, and reduce noise and competing demands.

4. Adapt

For misunderstanding or repetition, respond to the underlying meaning or emotion, use familiar words or visual supports, and avoid unnecessary correction.

5. Use activities

Personalize around routines, life roles, interests, culture, music, objects, and the best time of day. Record what worked so the next caregiver can begin well.

6. End well

Observe changes from the person's usual pattern. Escalate acute or concerning changes through normal clinical or supervisory channels; do not diagnose.

Understand communication in context

Communication is shaped by dementia, but also by hearing, vision, pain, fatigue, medication, unfamiliar routines, noise, language, culture, and the approach of the communication partner. Check the whole situation before attributing difficulty to dementia.

A sudden change from usual communication may need prompt clinical or supervisory escalation.

Approach and gain attention

Come into the person's field of view, identify yourself, use the preferred name, and wait for attention before giving information. Keep your posture and tone calm and adult-to-adult.

Avoid calling across a room, talking while turning away, or beginning care before the person understands what is happening.

Support understanding

Use familiar words and short, natural sentences. Present one idea or instruction, pause, and show or gesture when useful. Check understanding without quizzing.

Reduce television, corridor noise, glare, and competing conversations. Make sure hearing and visual supports are available.

Preserve expression and choice

Allow extra response time. Listen for key words and broader meaning. Offer two meaningful options rather than an open-ended demand or a long list.

Treat refusal as communication. Pause and explore discomfort, timing, fear, pain, privacy, or a different preferred approach.

Communicate during everyday care

Explain before touching or moving equipment. Break care into manageable steps and invite participation at each step. Maintain privacy and speak directly to the person even when family or colleagues are present.

Use familiar routines and objects to make the sequence easier to understand.

Respond to repetition and misunderstanding

Answer repeated questions calmly and consistently. A written or visual cue may help. Look for the emotion or need underneath the words.

Avoid unnecessary correction. When accuracy is essential for safety, give clear information without argument or humiliation.

Respond to distress or disengagement

Pause the task, lower your voice, reduce demands, validate the emotion, and offer time or space. Watch for pain, breathlessness, acute confusion, or other changes requiring escalation.

Do not use a preferred activity to pressure compliance. Connection and consent come before task completion.

Personalize communication

Record preferred name, language, life roles, routines, meaningful people, music, interests, distress signals, effective cues, and best times of day. Confirm information rather than stereotyping culture or generation.

A personalized aid works best when the communication partner knows how and when to use it.

Handover and reflection

Record observable facts: what was happening, what the person communicated, what you tried, and what helped. Avoid labels such as difficult or uncooperative.

Share useful strategies through approved records and handover so the next caregiver can begin from knowledge rather than trial and error.

Escalation and scope

Follow the organization's normal clinical and safeguarding pathways for acute confusion, pain, injury, breathing difficulty, marked drowsiness, sudden speech change, abuse concerns, or other significant changes.

These materials support communication practice. They do not replace assessment, professional training, policy, or clinical judgment.

Evidence behind this guide

Full references

Evidence note: Informed by the seven systematic reviews and evidence syntheses listed at sageandseed.org/full-references.html. Evidence for communication skills and meaningful interaction is more consistent than evidence for broader clinical outcomes.

This practical resource supports communication and engagement. It is not medical advice, therapy, clinical training, or certification. Follow local safeguarding and supervisory procedures.